

OPERATIONS & COMPLIANCE

Medical Director Oversight & Delegation

The delegation structure, standing orders and chart review that every clinical protocol depends on

DOCUMENT OPS-001	VERSION 1.0	EFFECTIVE January 2026	REVIEW CYCLE Annual
APPLIES TO Medical director, owner, practice manager	OWNER Medical Director	CLASSIFICATION Confidential	SUPERSEDES All prior versions

1 Purpose & Scope

This is the foundation document. Every clinical protocol in the practice operates under the delegation authority and standing orders defined here. It sets out the medical director's duties, the mechanism by which clinical tasks are delegated, the chart review obligation, and the annual re-authorization cycle that keeps all of it current.

READ THIS BEFORE ANYTHING ELSE

A signed clinical SOP is only meaningful if the person who signed it has a documented oversight relationship with the practice and the authority to delegate the tasks inside it. If OPS-001 is incomplete, every other protocol in your binder is floating.

LEGAL REVIEW REQUIRED

Medical director agreements, ownership structures and delegation arrangements carry corporate practice of medicine implications that differ sharply between states — including whether a non-physician may own the practice at all, and whether a management services organization is required. **This template must be reviewed by a licensed healthcare attorney in your state before execution.** It is designed to make that review faster, not to replace it.

2 Medical Director Agreement — Required Elements

A one-page contract naming a physician and a monthly fee is the arrangement most commonly found in practice and the one least likely to withstand scrutiny. The agreement should address each element below.

AGREEMENT CHECKLIST

Element	What it must specify
Parties and licensure	Full legal names, the physician's license number and state, specialty, and confirmation of good standing verified at execution.
Scope of oversight	Which service lines and locations the medical director is responsible for. Silence here is read as everything.
Duties	Protocol authorship and approval, standing orders, chart review, staff competency sign-off, adverse event review, and complaint involvement — each stated explicitly.
Time commitment	Defined hours or a defined cadence of activity. A retainer with no stated activity is the weakest possible arrangement.
Availability and response	How the medical director is reached during clinic hours, expected response time, and who covers absence, vacation and after-hours.
On-site presence	Whether physical presence is required for any service, and at what frequency — state-dependent and must be reconciled with your state's rules.
Compensation	Structure and basis. Fee arrangements tied to treatment volume or revenue raise fee-splitting and anti-kickback concerns in many jurisdictions — flag for legal review.
Term, renewal and termination	Including notice period and an orderly transition obligation so services do not continue unsupervised during a handover.
Insurance	Malpractice coverage carried by each party, limits, and tail coverage on termination.
Records access	Confirmation that the medical director may access any patient record at any time.

3 Standing Orders & Delegation

Standing orders are the mechanism that permits staff to perform clinical tasks without an order for each individual patient. They are specific, written, signed and dated — a general statement that the medical director approves the practice's activities is not a standing order.

Requirements for a valid standing order

- Names the specific procedure, medication or intervention authorized.
- Names the role authorized to perform it, and the minimum license or credential required.
- States the patient criteria under which it may be performed, and the exclusions.
- States the circumstances requiring escalation to the medical director before proceeding.
- Is signed and dated by the medical director, with a stated review date.
- Is reconciled against your state's scope of practice and supervision rules.

DELEGATION IS TASK-BY-TASK

Authorization is granted for a specific task to a specific role, not to a person in general. A registered nurse authorized to administer neurotoxin is not thereby authorized to assess candidacy, to treat a complication, or to operate a laser. Build the matrix in OPS-003 and have the medical director sign it line by line.

Annual re-authorization

- ☐ Every clinical protocol reviewed and re-signed by the medical director annually.
- ☐ Standing orders reviewed against current clinical guidance and any change in state rules.
- ☐ Delegation matrix reviewed against current staffing and license status.
- ☐ Superseded versions retained rather than destroyed — you may need to show what was in force on a given date.
- ☐ Review date, participants and outcomes recorded in the oversight log.

4 Chart Review

Chart review is the activity that converts a nominal oversight relationship into a demonstrable one. It is also the single most commonly omitted medical director duty.

Element	Standard
Cadence	Set a defined interval — monthly is common. Record it in the agreement rather than leaving it to judgement.
Sampling method	Define how charts are selected: a percentage of total volume, a fixed number per provider, plus all adverse events and all new providers during their supervised period.
What is reviewed	Candidacy documentation, consent, treatment record completeness, product and lot recording, aftercare, and follow-up closure.
Findings	Recorded in an oversight log with the charts reviewed, issues identified, and corrective actions assigned.
Follow-through	Corrective actions verified complete at the next review. An unclosed action is worse than no review.
Escalation	Any pattern suggesting a competency gap triggers the process in OPS-003 rather than an informal conversation.

5 Oversight Records to Retain

- ☐ Executed medical director agreement, plus all superseded versions.
- ☐ Current license verification with the date checked and re-verification at each renewal.
- ☐ Malpractice certificate of insurance for the medical director, current and on file.
- ☐ Every signed clinical protocol with its authorization page completed.
- ☐ Signed delegation matrix, current version plus prior versions.
- ☐ Chart review log with findings and closed corrective actions.
- ☐ Adverse event reviews with the medical director's documented involvement.
- ☐ Annual protocol review sign-off for each year the practice has operated.

THE FIVE-MINUTE TEST

One named person should be able to produce all eight items above within five minutes of being asked. If that is not true today, that gap is your first corrective action.

6 Quick Reference

QUICK REFERENCE CARD

Oversight essentials

1. **Agreement** — executed, specific duties, verified license, insurance on file.

2. **Standing orders** — task-specific, role-specific, signed and dated.

3. **Delegation matrix** — line-by-line, reconciled to state scope rules.

4. **Chart review** — defined cadence and sampling, findings logged.

5. **Corrective actions** — assigned with an owner and closed out.

6. **Annual re-authorization** — every protocol re-signed, prior versions retained.

7. **Legal review** — attorney sign-off before execution, not after.

Print, laminate and post at the point of care

7 Medical Director Authorization

This protocol is adopted as a standing order for the practice named below. By signing, the medical director confirms the protocol has been reviewed against current clinical guidance, applicable state scope-of-practice rules, and the training level of the staff authorized to perform it.

PRACTICE NAME

LOCATION / SITE

MEDICAL DIRECTOR — PRINTED NAME & CREDENTIALS

LICENSE NUMBER & STATE

SIGNATURE

DATE ADOPTED / NEXT REVIEW DATE

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